Paranoid Personality Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Paranoid personality disorder (PPD) is a long-standing pattern of distrust and suspicion in which other people's motives are read as harmful, beginning by early adulthood and showing up across many situations (American Psychiatric Association [APA], 2022). It belongs to **Cluster A** (odd or eccentric) with schizoid and schizotypal personality disorders. Unlike delusional disorder or schizophrenia, the suspicion is not held with psychotic certainty.

How common: in a large U.S. national survey, 4.41% of adults met criteria for PPD, making it one of the more common personality disorders (Grant et al., 2004). Rates vary widely depending on how studies assess personality disorders (Volkert et al., 2018).

A neglected disorder. PPD is often confused with other diagnoses and met with pessimism about treatment; it receives little clinical attention compared with its frequency (Carroll, 2009; Lee, 2017).

Counselor framing: these clients rarely come asking for help with trust. They come for depression, anxiety, conflict at work, or marriage problems, and they watch the counselor closely for any sign of betrayal.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis. A pervasive distrust and suspiciousness, beginning by early adulthood, with **4 or more** of:

- Suspects, without enough basis, that others are **exploiting, harming, or deceiving** him or her.
- Preoccupied with unjustified **doubts about the loyalty or trustworthiness** of friends or associates.
- **Reluctant to confide** in others for fear the information will be used against him or her.
- Reads **hidden demeaning or threatening meanings** into harmless remarks or events.
- Persistently **bears grudges** and is unforgiving of insults, injuries, or slights.
- Perceives **attacks on character or reputation** that others do not see, and is quick to react angrily or counterattack.
- Has recurrent, unjustified **suspicions about the faithfulness** of a spouse or partner.

Rule out: the pattern must not occur only during schizophrenia, a mood disorder with psychotic features, or another psychotic disorder, and must not be due to a medical condition or substance use. Also consider realistic mistrust shaped by past mistreatment, discrimination, or a dangerous environment (APA, 2022).

When counselors must refer (general clinical consensus): refer for psychiatric evaluation if suspicions become fixed and unshakable (possible delusions), if there are threats of violence or plans for revenge, or if substance use is heavy. Assess risk to others, including partners, with care (Carroll, 2009).

3. What works: treatment and the evidence

Bottom line for class: there are almost no treatment trials for PPD alone. Treatment rests on a patient, respectful alliance and on cognitive and schema-focused approaches studied across personality disorders.

Evidence overview (honest effect sizes):

- **Schema therapy:** in a multicenter trial of 323 patients with avoidant, dependent, obsessive-compulsive, paranoid, histrionic, or narcissistic personality disorder, schema therapy (which targets long-standing core beliefs about self and others) led to recovery more often than usual care or another psychotherapy (Bamelis et al., 2014). Few participants had PPD, so results for PPD specifically are uncertain.
- **No PPD-specific trials:** reviews describe the treatment evidence as very limited and rely on clinical guidance (Carroll, 2009; Lee, 2017).
- **Medication (referral notes):** no medication is approved for PPD. Medication may treat co-occurring depression, anxiety, or brief psychotic symptoms; these clients may be suspicious of medicines too, so clear explanation matters.

Cognitive work with PPD: how it is used (Bamelis et al., 2014; Carroll, 2009):

- **Build trust slowly.** Be transparent, predictable, and honest about notes, confidentiality, and limits. Avoid pushing for disclosure too early or confronting suspicions head-on.
- **Identify and challenge core beliefs:** "People are out to get me," "If I let my guard down, I'll be hurt," "Any kindness hides a motive," "Forgiving means letting them win."
- **Test interpretations:** list other explanations for an ambiguous event, run small behavioral experiments in trusting, and track what actually happens.

4. Religion, spirituality, and Christian integration: what the evidence shows

Disorder-specific research. We found **no study of religion or spirituality in paranoid personality disorder** and no faith-integrated treatment trial (literature search, November 1, 2026).

Related evidence. Across 49 studies, positive religious coping (trusting God, seeking His help) went with better adjustment to stress, and negative religious coping (feeling punished or abandoned by God) with worse (Ano & Vasconcelles, 2005). Across 97 outcome studies, therapy adapted to clients' religious beliefs worked about as well as standard therapy for distress and produced added spiritual benefits (Captari et al., 2018).

Honest summary for teaching:

1. A trusting alliance and cognitive, schema-focused work are the best available approach. Faith integration supports them; it does not replace them.
2. There is no research on faith and PPD. Draw on general evidence about religious coping and religiously adapted therapy.
3. Suspicion can extend to churches, pastors, and God. Never pressure trust; earn it.

5. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within cognitive, alliance-centered care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for spiritual forms of mistrust: "The people at church are talking about me," "God lets people hurt me," "If I forgive, they'll do it again," "I can't trust anyone, not even God" (Ano & Vasconcelles, 2005).
- **Challenge (clinical).** Separate wise caution from blanket suspicion. Weigh the evidence for each specific fear, and practice small, safe risks of trust with people who have proven reliable (Bamelis et al., 2014).
- **Replace (pastoral).** Offer Scripture of God's protection and of forgiveness that releases bitterness without denying wrong:
  - "When I am afraid, I put my trust in You. In God, whose word I praise—in God I trust; I will not be afraid. What can man do to me?" (Psalm 56:3–4)
  - "I will lie down and sleep in peace, for You alone, O LORD, make me dwell in safety." (Psalm 4:8). Security that does not depend on constant watchfulness.
  - "Love is patient, love is kind… it is not easily angered, it keeps no account of wrongs." (1 Corinthians 13:4–5). Letting go of the grudge ledger.
  - "Get rid of all bitterness, rage and anger, clamor and slander, along with every form of malice. Be kind and tenderhearted to one another, forgiving each other just as God in Christ forgave you." (Ephesians 4:31–32). Forgiveness is not the same as trusting an unsafe person.
- **Faith community (clinical).** Encourage one or two steady, discreet relationships at church rather than large groups. Gossip, broken confidences, or sudden changes can confirm the client's worst fears.
- **Ministers:** be consistent, keep confidences, avoid arguing over every suspicion, and refer when suspicion becomes fixed belief or when anyone may be at risk.

6. Quick classroom takeaways

- PPD is long-standing, pervasive distrust (4 of 7 criteria) without psychotic certainty. It affects about 4% of U.S. adults but is rarely the reason clients seek help.
- No PPD-specific trials exist; schema and cognitive therapy across personality disorders, built on a careful alliance, are the best evidence.

- Replace "people are out to get me" with the truth that God is a refuge, and with forgiveness that frees without requiring blind trust.

Sources note

All 8 references were checked against Crossref, OpenAlex, and PubMed/Europe PMC records (Nov 1, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record.

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